

August 12, 2026

Absolute Total Care (Centene)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Stanford Windler (MRN MUSC9546840), Claim MUSC-20260420-1B1C-2

Patient	Stanford Windler (DOB 1941-08-06)
MRN	MUSC9546840
Member ID / Group	ABS330201565 / GRP-63437
Claim number	MUSC-20260420-1B1C-2
Date of service	2026-04-20
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$569.26
Denial code	CARC 151 / RARC N362 — Payer deems the information submitted does not support this many/frequency of services.
Appeal deadline	2026-08-19

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this first-level appeal on behalf of Stanford Windler (Member ID: ABS330201565, MRN: MUSC2704695, Date of Birth: August 6, 1941) regarding the denial of Claim MUSC-20260420-1B1C-2 for services rendered on April 20, 2026, at MUSC Health Rutledge Tower Ambulatory Care. The claim was submitted on April 26, 2026, and denied by Absolute Total Care on May 21, 2026. The denied service is one unit of CPT 99214, an established patient office visit of moderate complexity, billed at \$569.26. The payer denied the claim in full under CARC 151 ("Payer deems the information submitted does not support this many/frequency of services") and RARC N362 ("The number of days or units of service exceeds our acceptable maximum"), with the accompanying remark: "Units billed exceed the plan's allowable frequency for this service." MUSC Health respectfully disputes this denial. Only one unit of CPT 99214 was billed for a single face-to-face encounter on a single date of service, and a frequency-based denial is not applicable under these circumstances.

CLINICAL BACKGROUND

Mr. Windler is an 85-year-old male with a complex, longstanding medical history managed at MUSC Health. His active conditions include coronary heart disease (I25.10), for which he carries prescriptions for simvastatin, amlodipine, nitroglycerin, and clopidogrel — all initiated in 2014 and currently active. He also carries active diagnoses of osteoporosis, gout, anemia, prediabetes, and obesity, and has a documented history of chronic sinusitis dating to 1952. The April 20, 2026, encounter was conducted by Dr. Elena V. Castellanos, MD (NPI 1881726354) in the on-campus outpatient hospital setting (Place of Service 22). The primary diagnosis for this visit was acute upper respiratory infection, unspecified (J06.9), with coronary heart disease (I25.10) coded as a secondary diagnosis — a clinically appropriate pairing given Mr. Windler's active cardiac medication regimen and the need to assess any respiratory illness in the context of underlying cardiovascular disease. The level of service, CPT 99214, reflects moderate complexity consistent with the management of an acute condition overlaid on multiple chronic, active comorbidities in an elderly patient.

BASIS FOR APPEAL

The denial rationale — that the number of units or frequency of service exceeds the plan's allowable maximum — is factually inapplicable to this claim. CPT 99214 was billed as one unit for one discrete outpatient encounter on April 20, 2026. A single evaluation and management service rendered on a single date of service cannot, by definition, constitute an excess frequency of that service on that date. CARC 151 and RARC N362 are appropriate when a provider submits multiple units of a service that is limited to one occurrence per day or per period, or when the same service is billed redundantly across overlapping claims. Neither circumstance applies here. The claim history in the case record does not reflect any other claim for CPT 99214 submitted for Mr. Windler on April 20, 2026, nor does the denial notice identify a conflicting claim or a specific frequency limit that this single unit allegedly exceeds. The denial therefore lacks a factual or coding basis. Furthermore, given Mr. Windler's age, active coronary heart disease, and multiple chronic conditions, an established-patient moderate-complexity visit is not only appropriate but represents the standard of care for managing an acute illness in a medically complex geriatric patient.

REQUESTED ACTION

MUSC Health respectfully requests that Absolute Total Care conduct a complete review of this appeal, reverse the denial of Claim MUSC-20260420-1B1C-2, and issue payment at the allowed amount of \$316.02 for the single unit of CPT 99214 rendered on April 20, 2026. Should the plan require additional clinical documentation, operative notes, or a peer-to-peer review with Dr. Castellanos, MUSC Health is prepared to provide those materials promptly through the secure provider portal in advance of the appeal deadline of August 19, 2026. We ask that this matter be resolved without further delay, as the service was medically necessary, correctly coded, and submitted in a timely and complete manner.

Respectfully submitted,

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record